

Executive Summary for Government Stakeholders

UAC Program Healthcare Capacity, Care Load & Flow Analysis

Purpose

This analysis provides a data-driven framework for monitoring the operational workload of the **Unaccompanied Alien Children (UAC) care system**, with emphasis on system load, transfers, HHS care, discharges, emerging pressure, and data quality.

The objective is to help government stakeholders move from **reactive monitoring to proactive capacity planning**.

The analysis uses the uploaded UAC operational dataset covering **January 12, 2023 through December 21, 2025**. After removing blank records, the dataset contains **720 populated reporting observations**.

1. Key Findings

HHS represents the primary active-care workload

The analysis estimates an average:

- **6,061 children in HHS care**
- **171 children in CBP custody**
- **6,233 children in combined system load**

This indicates that the majority of the active workload represented in the dataset occurs within the HHS care component rather than the CBP custody component.

Government implication: Resource planning should place substantial emphasis on HHS shelter, healthcare, case management, staffing, and discharge capacity rather than relying solely on border-intake indicators.

System workload is highly variable

The observed combined system load ranged from approximately:

2,002 → 11,762 children

The historical maximum occurred on **December 20, 2023**.

This wide range demonstrates that the UAC care system experiences significant changes in operational demand.

Government implication: Capacity planning should incorporate both baseline requirements and surge scenarios.

Transfers and discharges are not consistently balanced

Average daily activity was approximately:

- **129 transfers into HHS**
- **173 HHS discharges**

The resulting average net pressure was approximately:

–45 children per reporting observation

Overall, this indicates that discharge activity generally exceeded transfers across the available observations.

However, the aggregate average masks important short-term pressure periods.

Positive pressure occurred frequently enough to require monitoring

Approximately:

- **238 reporting observations** had positive net intake pressure.
- **475 observations** had negative net intake pressure.
- **7 observations** were approximately balanced.

Therefore, although the overall average indicates relief, the system experienced numerous periods in which incoming transfers exceeded discharges.

Government implication: A single monthly or annual average should not be used as the sole capacity indicator.

2. Strategic Risk Areas

Risk 1 — Short-term surges can be hidden by long-term averages

A negative overall net-pressure average does not guarantee that the system is operationally stable.

Several consecutive days of:

Transfers > Discharges

can cause the active HHS population to rise rapidly.

Recommended response

Implement automated monitoring of:

- 7-day pressure;
 - 14-day pressure;
 - current HHS census;
 - recent peak census; and
 - consecutive positive-pressure days.
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Risk 2 — Capacity data is incomplete

The dataset measures the number of children in care but does **not** provide facility-level capacity.

Therefore:

High care load $\neq$ confirmed facility overcapacity.

The analysis identifies **capacity pressure**, not actual physical capacity utilization.

Recommended response

Integrate:

- licensed bed capacity;
- currently available beds;
- occupied beds;
- staffing capacity;
- healthcare staffing ratios;
- surge beds;
- facility location; and
- facility-level utilization.

This would enable government agencies to transition from load monitoring to true capacity forecasting.

Risk 3 — Reporting gaps reduce situational awareness

The dataset contains **355 missing calendar dates** across the reporting period.

Missing observations must not be interpreted as zero activity.

Recommended response

Establish a centralized reporting-quality indicator showing:

Reported / Missing / Delayed / Corrected / Anomalous

for every reporting date.

Risk 4 — Flow anomalies require contextual investigation

There are **86 observations where transfers exceed same-day CBP custody**.

This should not automatically be classified as incorrect data because custody represents a stock while transfers represent a flow.

Recommended response

Create an operational review flag rather than automatically removing the records.

3. Recommended Government KPI Framework

KPI	Purpose	Recommended interpretation
Total System Load	Combined active workload CBP + HHS	
HHS Care Load	HHS operational workload	Primary care indicator
Net Intake Pressure	Transfer/discharge balance	Positive = accumulation
7-Day Pressure	Short-term trend	Detect emerging pressure
14-Day Pressure	Sustained trend	Detect persistent pressure
Backlog Pressure	Cumulative accumulation	Identify prolonged stress
Discharge Offset Ratio	Flow efficiency	Discharges ÷ transfers
Load Volatility	Stability	Daily workload variation
Reporting Coverage	Data reliability	Reported vs expected days

4. Proposed Early-Warning Framework

A government dashboard could use a four-level analytical alert system.

☐ GREEN — Stable

- HHS load stable or declining
- 7-day net pressure ≤ 0
- Discharges adequately offset transfers

Action: Normal operations.

☐ YELLOW — Emerging Pressure

- 7-day net pressure becomes positive
- HHS census begins increasing
- Transfer activity accelerates

Action: Increase monitoring and review staffing/shelter availability.

☐ ORANGE — Sustained Pressure

- Positive 7-day and 14-day pressure
- HHS census increasing
- Discharge offset deteriorating

Action: Prepare surge capacity and resource allocation.

● RED — High Operational Pressure

- Sustained positive pressure
- HHS census approaches historical highs
- Available capacity becomes constrained

Action: Activate established surge and contingency plans.

Important: These are analytical alert concepts, not official government thresholds. Actual thresholds should be calibrated against verified facility capacity and operational standards.

5. Policy Recommendations

1. Establish a centralized UAC capacity-monitoring dashboard

Provide decision-makers with a single view of:

CBP → Transfer → HHS → Discharge

rather than relying on disconnected operational reports.

2. Shift from point-in-time reporting to trend-based monitoring

Daily counts should be supplemented with:

- 7-day rolling averages;
- 14-day rolling averages;
- weekly changes;
- monthly trends; and
- historical percentile comparisons.

This makes emerging pressure easier to identify.

3. Strengthen surge-capacity planning

Historical peak workload should be incorporated into scenario planning.

Planning scenarios could include:

- +10% transfer volume;
- +20% transfer volume;
- 10% reduction in discharge throughput;
- simultaneous transfer increase and discharge slowdown.

This would allow policymakers to estimate how quickly the active care population could grow under adverse conditions.

4. Prioritize discharge capacity

Discharge throughput is an important pressure-release mechanism.

Monitoring should therefore include:

How effectively is the system converting incoming transfers into completed discharges?

The **Discharge Offset Ratio** can provide a simple operational signal.

5. Integrate capacity and workforce information

Future dashboards should combine child counts with:

- shelter capacity;
- healthcare staffing;
- case-management staffing;
- available beds;
- surge capacity; and
- geographic capacity.

This would allow agencies to identify **where** pressure exists rather than only **whether** pressure exists.

6. What Government Leaders Should Monitor

At an executive level, the dashboard should answer five questions:

1. How many children are currently under active care?

Total System Load

2. Is the workload increasing or decreasing?

System Load Trend

3. Are incoming transfers exceeding discharges?

Net Intake Pressure

4. Is pressure temporary or sustained?

7-Day and 14-Day Rolling Pressure

5. Can we trust the operational picture?

Reporting Coverage + Data Quality Flags

7. Strategic Conclusion

The analysis demonstrates that UAC operational data can support a more proactive approach to government capacity management.

The principal finding is not simply that workload rises and falls. Rather, **the timing and persistence of those changes matter.**

A system can experience an apparently manageable average workload while simultaneously entering a period of rapidly increasing pressure. Government decision-making therefore benefits from monitoring **flow dynamics in addition to population counts.**

The recommended model is:

Monitor the stock → monitor the flow → identify sustained pressure → compare against actual capacity → activate appropriate planning response.

The immediate priority should be establishing a centralized analytical dashboard that integrates **HHS census, CBP custody, transfers, discharges, rolling pressure, historical peaks, capacity data, and reporting quality.**

With facility-level capacity and workforce information added, this framework can evolve into a **predictive early-warning and surge-planning system** capable of supporting healthcare infrastructure, shelter operations, staffing decisions, and humanitarian response.

Executive takeaway

The UAC care system should be managed as a dynamic pipeline rather than a static population. The most valuable early-warning signals are not simply how many children are currently in care, but whether transfers are persistently exceeding discharges, whether the active HHS population is rising, and whether available capacity is sufficient to absorb the emerging workload.